

23. Mangione CM, Lee PP, Gutierrez PR, et al. Development of the 25-item National Eye Institute Visual Function Questionnaire. *Arch Ophthalmol*. 2001;119:1050-1058.
24. Ishii K, Kabata T, Oshika T. The impact of cataract surgery on cognitive impairment and depressive mental status in elderly patients. *Am J Ophthalmol*. 2008;146:404-409.
25. Kovac B, Vukosavljevic M, Djokic KJ, et al. Validation and cross-cultural adaptation of the National Eye Institute Visual Function Questionnaire (NEI VFQ-25) in Serbian patients. *Health Qual Life Outcomes*. 2015;13:142.
26. Mak ST, Wong AC. Vision-related quality of life in corneal graft recipients. *Eye (Lond)*. 2012;26:1249-1255.

Footnotes and Financial Disclosures

Originally received: August 7, 2016.

Final revision: April 12, 2017.

Accepted: April 13, 2017.

Available online: May 16, 2017.

Manuscript no. 2016-33.

¹ VA Tennessee Valley Healthcare System, Nashville, Tennessee.

² Vanderbilt University School of Medicine, Nashville, Tennessee.

³ W.K. Kellogg Eye Center, University of Michigan Health System, Ann Arbor, Michigan.

⁴ St. Louis VA Medical Center, St. Louis, Missouri.

⁵ Washington University School of Medicine, St. Louis, Missouri.

⁶ Michael E. DeBakey VA Medical Center, Houston, Texas.

⁷ Cullen Eye Institute, Baylor College of Medicine, Houston, Texas.

⁸ VA Boston Healthcare System, Boston, Massachusetts.

⁹ Boston University School of Medicine, Boston, Massachusetts.

¹⁰ Harvard Medical School, Boston, Massachusetts.

¹¹ Department of Defense/VA Vision Center of Excellence, Arlington, Virginia.

Presented as a paper at: the American Academy of Ophthalmology Annual Meeting, October 18–21, 2014, Chicago, Illinois, and won the “Best Paper” award for the cataract section.

Financial Disclosure(s):

The author(s) have no proprietary or commercial interest in any materials discussed in this article.

Financial Support: U.S. Department of Veterans Affairs.

Author Contributions:

Conception and design: Shekhawat, Stock, Baze, Daly, Vollman, Lawrence, Chomsky

Data collection: Shekhawat, Stock, Baze, Daly, Vollman, Lawrence, Chomsky

Analysis and interpretation: Shekhawat, Stock, Chomsky

Obtained funding: Not applicable

Overall responsibility: Shekhawat, Chomsky

Abbreviations and Acronyms:

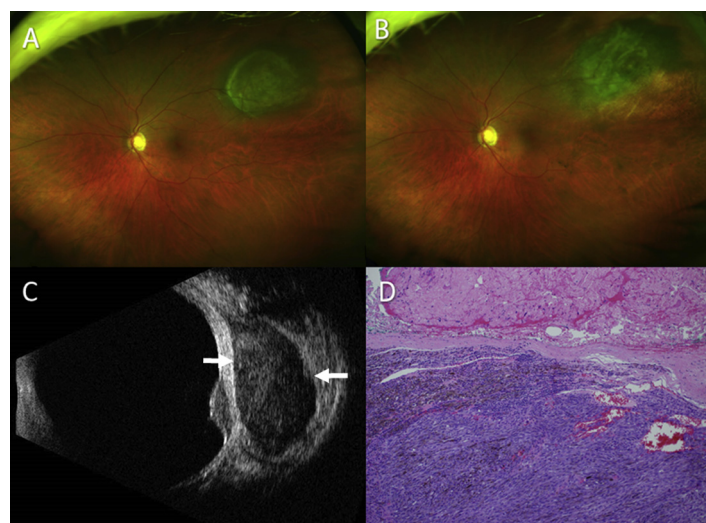
BCVA = best-corrected visual acuity; **CC** = cortical cataract; **logMAR** = logarithm of the minimum angle of resolution; **NEI-VFQ** = National Eye Institute Visual Function Questionnaire; **NSC** = nuclear sclerotic cataract; **OSOD** = Ophthalmic Surgical Outcome Data; **PSC** = posterior subcapsular cataract; **SD** = standard deviation; **VHA** = Veterans Health Administration; **VRQoL** = vision-related quality of life.

Correspondence:

Nakul S. Shekhawat, MD, MPH, 1000 Wall Street, Ann Arbor, MI 48105.

E-mail: nakul.shekhawat@vanderbilt.edu.

Pictures & Perspectives



Massive Extraocular Extension of Choroidal Melanoma after Brachytherapy

A 63-year-old woman presented with a pigmented choroidal tumor (Fig 1A), which progressively enlarged in thickness, from 2.3 to 3.6 mm in 6 months. Systemic work-up showed multiple lung nodules, which biopsy demonstrated adenocarcinoma. Fine-needle biopsy of the choroidal mass revealed melanoma. Patient underwent treatment with brachytherapy. Eighteen months later, the tumor regressed to 1.8 mm in thickness (Fig 1B). On ultrasound, there was a low reflective extraocular lesion, suggestive of extension of the choroidal melanoma (Fig 1C, arrows). Histopathologic analysis displayed mixed-type melanoma with epithelioid and spindle cells (Fig 1D). Tumor recurrence after brachytherapy can occur extra-sclerally and can be detected by ultrasound (Magnified version of Fig 1A-D is available online at www.aaojournal.org).

BERNADETE AYRES, MD

VICTOR ELNER, MD

HAKAN DEMIRCI, MD

Department of Ophthalmology and Visual Sciences, W. K. Kellogg Eye Center, University of Michigan, Ann Arbor, Michigan